

Patient ID: PHolstPeriod: February 22 to April 26, 2024Report Date: April 27, 2024Coverage: 1432/1537h (93%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 65 days from Feb 22 to Apr 26

HR Breathing

g M a	Low Heart Rate Feb 22 to Mar 07 (15d)	Low Heart Rate Mar 10 to Mar 24 (15d)	Heart Rate Mar 31 to Apr 02	Low Heart Rate Apr 12 to Apr 17 (6d)	Low Heart Rate Apr 21 to Apr 25 (5d)	a t e

Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	55 bpm	44 bpm	92 bpm
Breathing	17 brpm	11 brpm	32 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

86 episodic events Detected, spanning 5 days total. Conditions: Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Heart Rate. **Priority grade: Medium**

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
86	124h	Low Heart Rate	1	93%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 23	11 PM to 12 AM	1h	1	Very High Heart Rate	113	111	113	Check temp, hydration, pain; review for arrhythmia or infection
Mar 23	11 PM to 12 AM	1h	1	High Breathing	30	11	38	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 22	5 PM to 6 PM	56h	38	Low Heart Rate	43	40	77	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 10	6 AM to 8 AM	20h	15	Low Heart Rate	43	40	77	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 27	12 AM to 2 AM	5h	4	Low Heart Rate	43	40	68	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 31	5 AM to 8 AM	9h	5	Low Heart Rate	44	40	66	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

+ 2 additional condition(s) across Low Heart Rate; details in monitoring data.

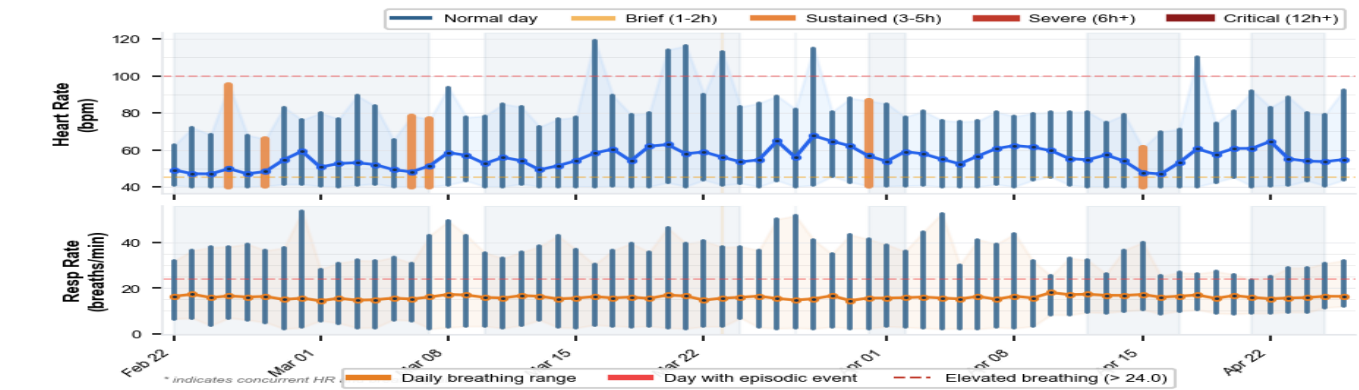
The P5 to P95 heart rate spread was 26 bpm (44 to 70), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 25 brpm, peak 39 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Elevated Heart Rate (avg 98 bpm, peak 115 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Heart Rate (avg 106 bpm, peak 119 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Breathing (avg 30 brpm, peak 38 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

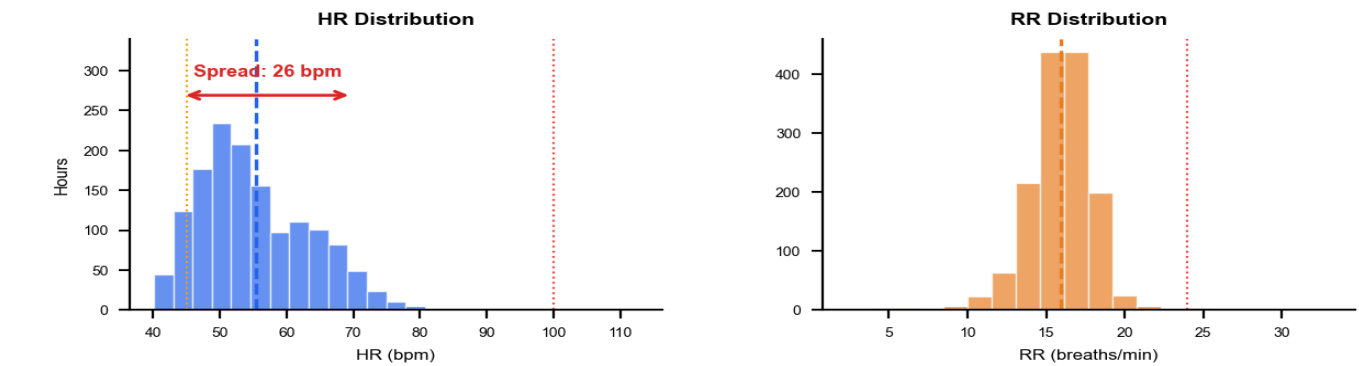
Clinical Pattern Observations:

- 1. **High variability:** HR spread 26 bpm (43 to 70).
- 2. **Clustered pattern:** Episodic events on 6 of 65 days (9%).
- 3. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.

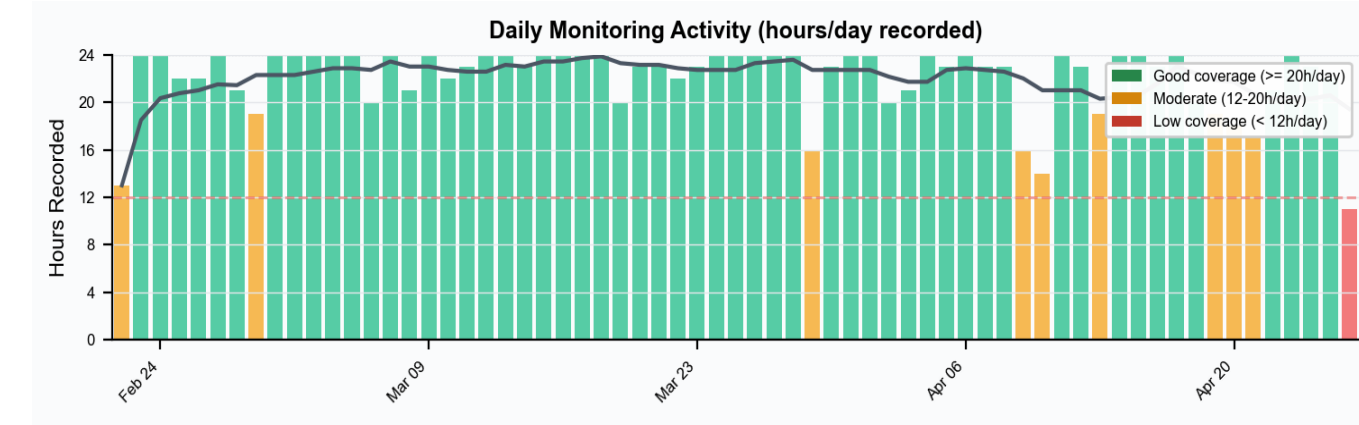


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.